

BAYCARE RHEUMATOLOGY

BayCare Medical Center
1725 E. Lake Rd, Suite 400, Palm Harbor, FL 34685
(727) 789-4400 | Fax: (727) 789-4411

OFFICE VISIT NOTE

Patient Name: Aaron Nikolaos Garcia
Date of Birth: December 2, 1967
Age: 58

MRN: BCR-018247

Insurance: BCBS of Florida, BCB-FL-GRP-88472931
Visit Date: February 18, 2026
Visit Type: Follow-up, Established Patient (Gout Management)
Provider: Lisa Fernandez, MD, FACR
Board Certified, Rheumatology
BayCare Rheumatology, Palm Harbor

REASON FOR VISIT

Follow-up visit for gout management following acute flare in January 2026.

HISTORY OF PRESENT ILLNESS

Mr. Garcia is a 58-year-old Greek-American male with a history of gout, first diagnosed 2020. He presents approximately five weeks after an acute gout flare affecting the right first metatarsophalangeal joint (right big toe), which began January 10, 2026. He reports the flare lasted approximately five days, with peak pain on days two and three. He initiated Colchicine 0.6mg on day one as instructed, which he keeps at his restaurant desk. He used Ibuprofen 400mg twice daily for pain management. He reports full resolution by January 16, 2026.

On inquiry, he identifies the likely trigger as consumption of shellfish (shrimp) at a holiday gathering in late December 2025. He typically avoids shellfish consistently but made an exception given the social context. He acknowledges this was his third consecutive trigger involving shellfish-related or organ meat-heavy meals at year-end events. He is frustrated and understands the pattern now clearly.

He denies any current joint pain or tenderness. He is taking Allopurinol 300mg daily without interruption. He has not self-discontinued Allopurinol at any point, which he confirms specifically.

VITAL SIGNS

Blood Pressure: 140/88 mmHg
Heart Rate: 76 bpm

Weight: 218 lbs

Temperature: 98.5 F

RELEVANT LABS (from Quest Diagnostics, drawn January 28, 2026)

Uric Acid: 5.8 mg/dL (Reference: 3.5-7.2; target for gout control: under 6.0)

Creatinine: 0.94 mg/dL (Normal; Allopurinol dose monitoring)

eGFR: 82 mL/min (Normal; no dose adjustment required)

PHYSICAL EXAMINATION

General: Well-appearing male in no acute distress. Ambulatory without difficulty.

Right foot: No erythema, no warmth, no swelling of the right first MTP joint.

Full range of motion, non-tender on palpation. Fully resolved.

Left foot: Unremarkable.

Other joints: No synovitis noted in knees, wrists, or elbows.

Skin: No tophi noted on ears, elbows, or fingers.

ASSESSMENT

1. Gout Tophaceous Disease - Stable (M10.071)

- Uric acid 5.8 mg/dL, within target range (under 6.0).
- January 2026 flare was triggered by shellfish ingestion, consistent with prior pattern. This is the third identifiable trigger event in a row involving shellfish or purine-heavy foods at social occasions.
- Allopurinol 300mg is providing adequate urate-lowering effect. No dose change indicated at this time.
- No tophi on examination.

PLAN

1. Continue Allopurinol 300mg once daily. Do not discontinue even during a flare.

2. Colchicine 0.6mg PRN (take as soon as flare begins, per existing prescription). Keep supply at restaurant as patient has been doing. Patient has adequate refills.

3. Dietary counseling reinforced:

- Absolute avoidance of shellfish (shrimp, lobster, crab, clams). Patient agreed. He understands this is non-negotiable given his response pattern.
- Limit red meat to two to three servings per week. Lamb is higher in purines; limit to one serving on days when it is consumed at the restaurant.
- Limit alcohol to two drinks maximum per sitting. Wine is preferable to beer. No organ meats.
- Increase water intake to at least 2 liters per day. Patient runs warm kitchens and tends to dehydrate during service; this likely contributed.
- Fasting periods (Lenten season) noted as likely to reduce flare frequency given the shift toward vegetables, legumes, and fish without shellfish.

4. Ibuprofen for pain during flare is acceptable given normal renal function.

Avoid chronic NSAID use given diabetes and hypertension context (discussed with PCP).

5. Follow-up: 6 months or sooner if flare occurs. Return appointment scheduled for October 2026

6. Coordinating with Dr. Robert Chen (PCP) on NSAID caution and overall cardiovascular risk profile.

MEDICATIONS (Active, rheumatology-managed)

1. Allopurinol 300mg - once daily, morning (urate-lowering therapy)
2. Colchicine 0.6mg - PRN for acute gout flare (keep at restaurant desk)

No new prescriptions written today.

Provider Signature: Lisa Fernandez, MD, FACR Date: February 18, 2026

NPI: 1720483916

This note is provided for patient reference. The full encounter record is maintained in the BayCare patient portal. For questions, call (727) 789-4400.